



PSYCHOTHERAPY

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PSYCHOTHERAPY

A process which attempts to help the patient relieve symptoms, resolve problems or seek personal growth through a structured relation (ex. Specified goals and methods) with a trained professional therapist.

The therapist may be a psychiatrist, psychologist, a nurse etc...



TYPES OF PSYCHOTHERAPY

A. According to format

1. individual therapy
2. group therapy
3. family therapy
4. marital therapy
5. community or milieu therapy

B. According to content (the applied personality theory)

1. Supportive
2. Expressive (dynamic or insight oriented)
3. Behavioral
4. Biofeedback
5. Behaviouralcognitive-behavioral
6. Experiential (humanistic approach)
7. Rehabilitation and activity therapies

SUPPORTIVE PSYCHOTHERAPY

Definition and aims

It is a form of therapy that deals with conscious conflicts and current problems.

It helps to support the patients and help them to:

1. Relieve symptoms and resolve problems
2. Regain equilibrium and maintain stability
3. Achieve better adaptation, coping and functioning.

SUPPORTIVE PSYCHOTHERAPY/ INDICATIONS

1. Crisis, acute distress or acute adjustment disorders
2. Chronic or handicapped patients (chronic schizophrenia, mood or personality disorders)
3. Patients who do not need or do not fit (lacking ego strength or intellectual ability) for deeper expressive therapy.

SUPPORTIVE PSYCHOTHERAPY/ TECHNIQUES

- Establishing an empathic understanding relation (therapeutic alliance)
- Active listening
- Reassurance and encouragement
- Suggestion, advice, persuasion
- Clarification and explanation (education) e.g. regarding symptoms, interpersonal problems and ways of coping

SUPPORTIVE PSYCHOTHERAPY/ TECHNIQUES

- Strengthening useful defences
- Suppression of unwanted conflicts
- Improving ego strength and functioning (reality testing, autonomy etc...)
- Environmental manipulation and modification
- Help in improving insight

SUPPORTIVE PSYCHOTHERAPY

➤ **Duration**

according to the patient's needs, it could be a single session or prolonged to many years

➤ **Format**

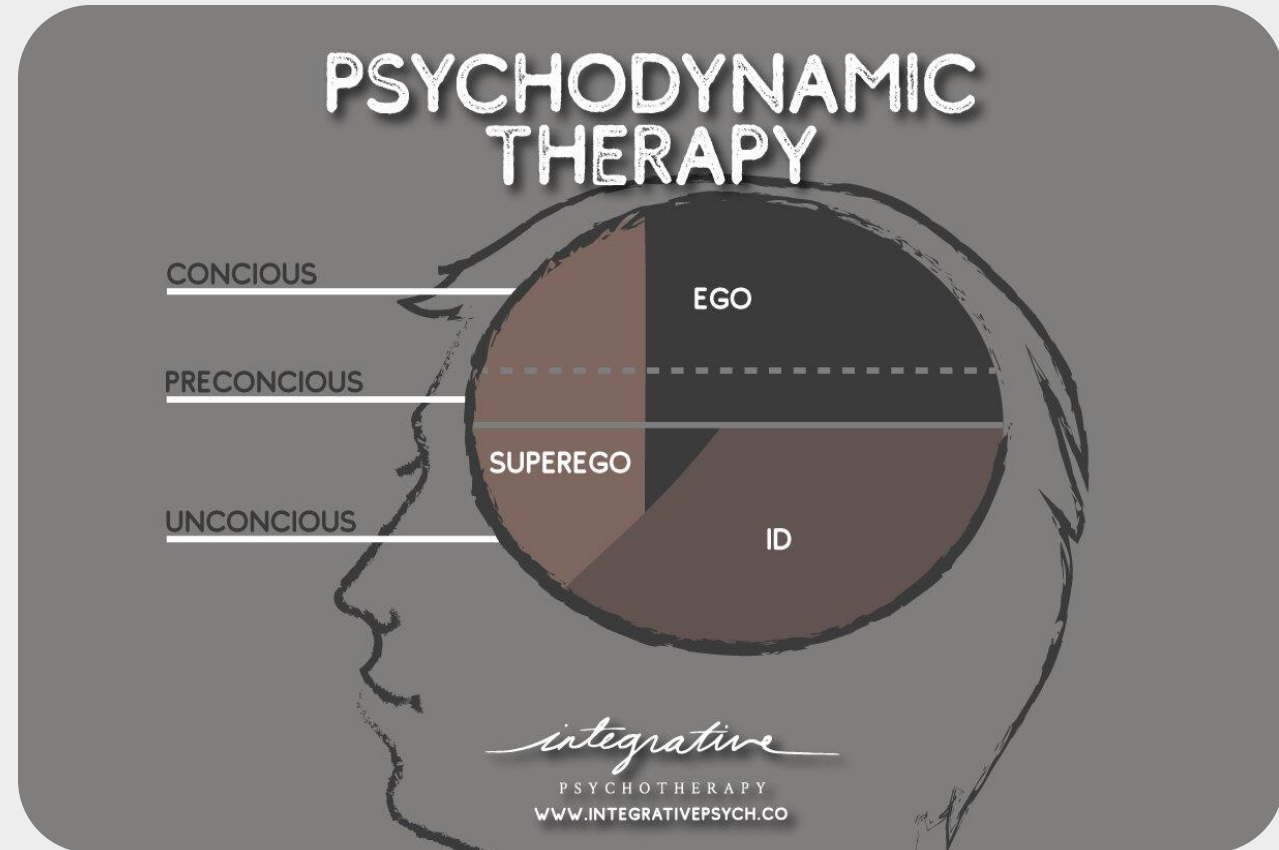
it is usually individual but can be used in all formats

➤ **Complications**

mainly dependency on the therapist

PSYCHODYNAMIC (EXPRESSIVE) THERAPY

A group of deep therapies that aim at symptoms resolution as well as producing positive fundamental changes in the patients' character or personality



PSYCHODYNAMIC (EXPRESSIVE) THERAPY/ ACHIEVED THROUGH

- Uncovering unconscious conflicts and shifting them to the conscious awareness of the patient
- Help the patient resolve the conflicts and correct deficits through understanding and insight
- Induce change motivated by insight
- Corrective relational and emotional experiences with the therapist

TYPES OF PSYCHODYNAMIC THERAPY

- **Classical psychoanalysis**
- **Psychoanalytic oriented models**
- **Short-term models**
- **Object relation**
- **Self-psychology models**

PSYCHODYNAMIC THERAPY

- Enough ego strength to tolerate the experience of change and growth
- The patients should be capable of expressing their thoughts and emotions
- It can be long-term (years), but short term models are more commonly used.

Table 2. Essential Operations of Psychodynamic Psychotherapy

Accepting	The therapist affirms the patient's past and present subjective experience.
Understanding	The therapist appreciates both the conscious and the unconscious contributions to the patient's emotional problems.
Explaining	The therapist expresses, through interpretations, his or her understanding to the patient.

Source: Adapted from Ornstein PH, Ornstein A: Clinical understanding and explaining: the empathic vantage point, in Progress in Self Psychology, vol 1. Edited by Goldberg A. New York, Guilford, 1985, pp 43–61

PSYCHODYNAMIC THERAPY/ SHORT-TERM TECHNIQUES CHARACTERISTICS

1. time-limited.

Ex. Weekly sessions for 3-6 months

2. Problem focused.

Ex. Focus on limited key aspects of the patient's psychopathology

Transference & Countertransference in Psychoanalytic Theory



Transference

The client unconsciously projects feelings about significant people in their life onto the therapist

Countertransference

This is the therapist's emotional reaction to the client, which may relate to a triggering of the therapist's own unresolved issues

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BEHAVIORAL THERAPIES

Behavioral therapy is based on the theory that symptoms are persistent maladaptive behaviors acquired by conditioning or learning.

- Therapy consists of “**deconditioning**” or “**unlearning**” of such behavioral habits and “**re-learning**” new adaptive behaviors

Ex. phobia

TECHNIQUES OF BEHAVIORAL THERAPY

1. systematic desensitisation

2. Graded exposure

3-Flooding

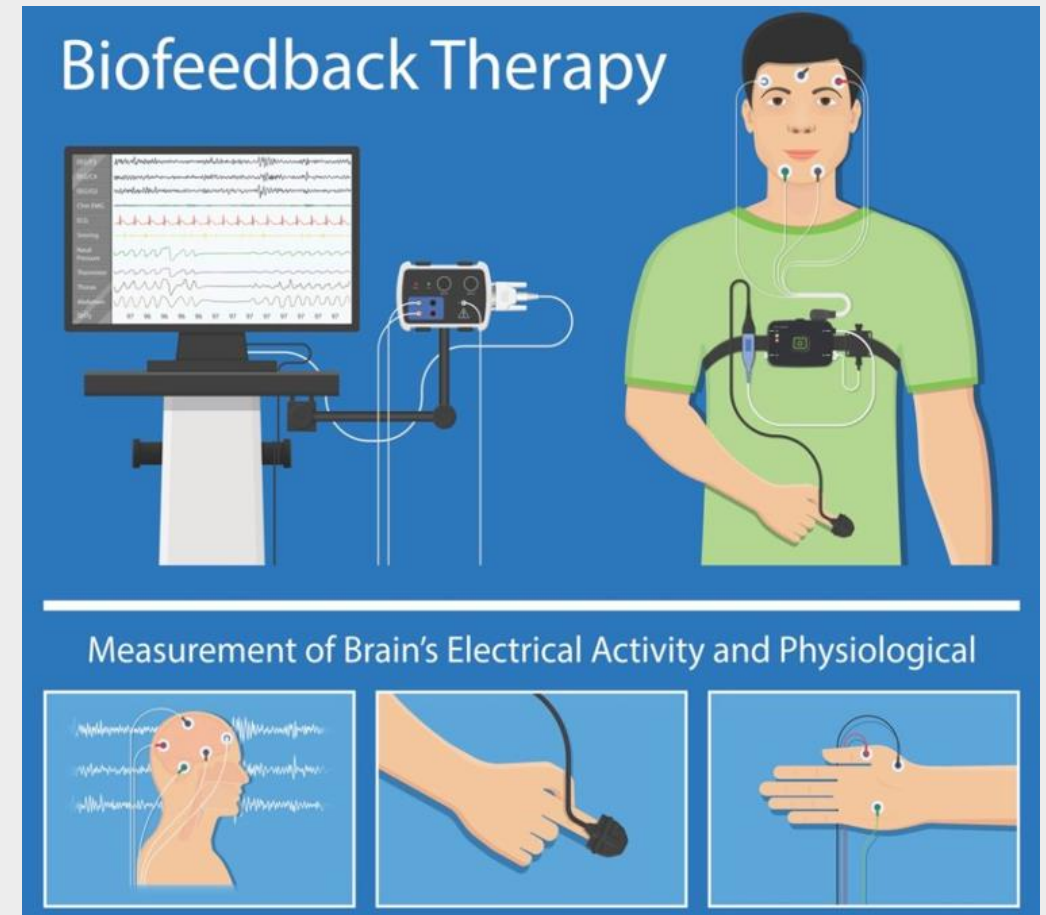
Note: the above techniques are useful in phobias, obsessive-compulsive disorders and some sexual disorders (Pedophilic disorder. Pedophilic disorder is a condition where you feel a persistent sexual attraction toward a minor, Exhibitionistic disorder, Voyeuristic disorder, Sexual sadism disorder, Sexual masochism disorder, Frotteuristic disorder, or Fetishism.).

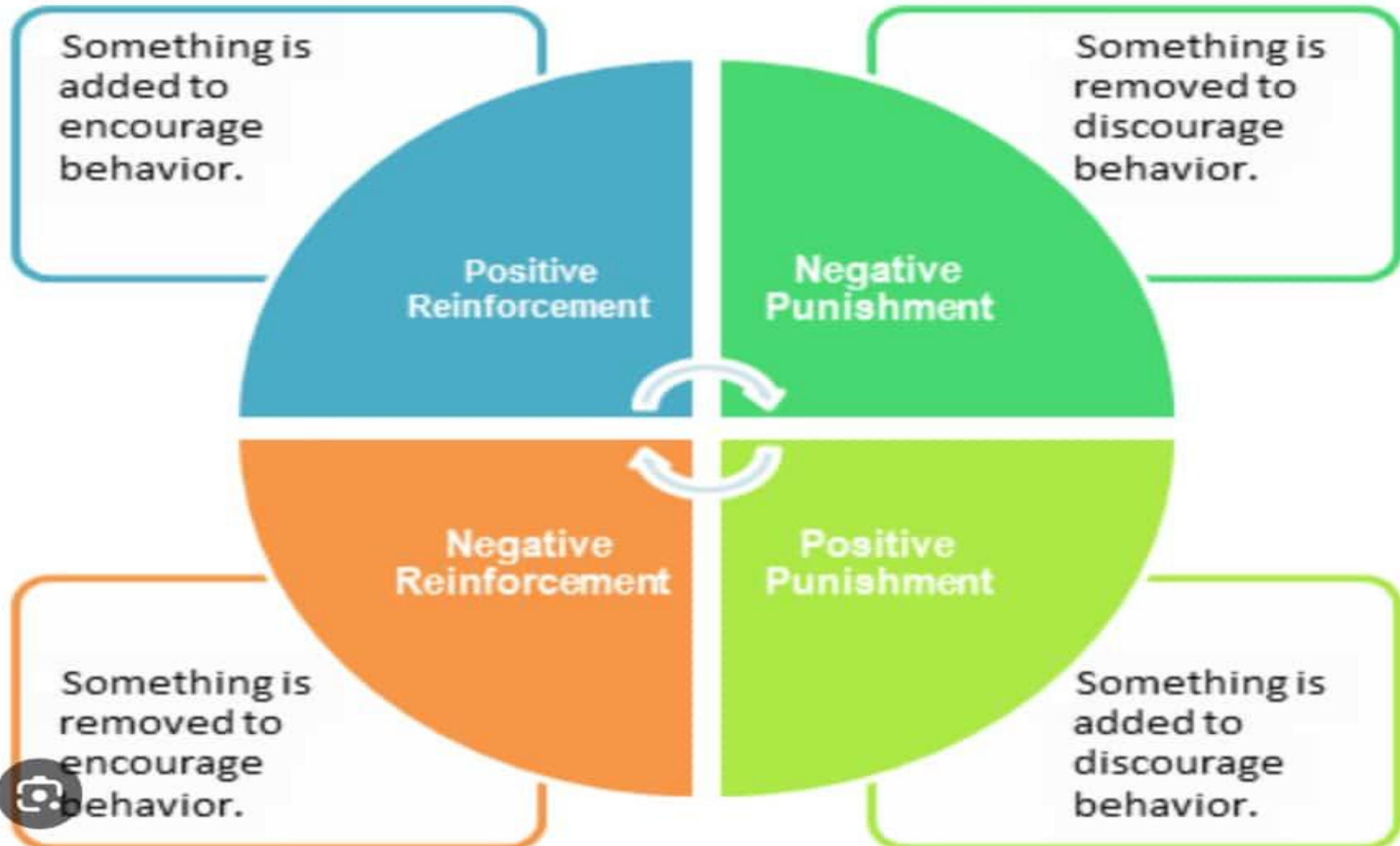
TECHNIQUES OF BEHAVIORAL THERAPY, CONTD...

4. **Aversive conditioning** (in treating alcoholism (disulfiram))
5. **Positive reinforcement (reward)**
for chronic patients (schizophrenia, autistic disorders, M.R. ...)
6. **Participant modelling**

BIOFEEDBACK

It is based on the idea that physiological functions (BP, HR, muscle tension etc...) which are not controlled voluntarily can be brought under voluntary control through operant conditioning if a person is provided with feedback information about these functions.





METHODS OF BIOFEEDBACK

- Information from measuring devices (**BP, skin temperature or galvanic response, EMG, EEC ...**) is transformed by electronic instruments into signals that can be perceived by the patient (ex. A sound or flashing light)
- On receiving these signals, the person can be trained to control those functions and reach a related euthymic mental state.

INDICATIONS OF BIOFEEDBACK

Used in the management of many psychosomatic conditions

Including hypertension arrhythmias, migraine, tension headache etc...



COGNITIVE-BEHAVIORAL THERAPY

Theory

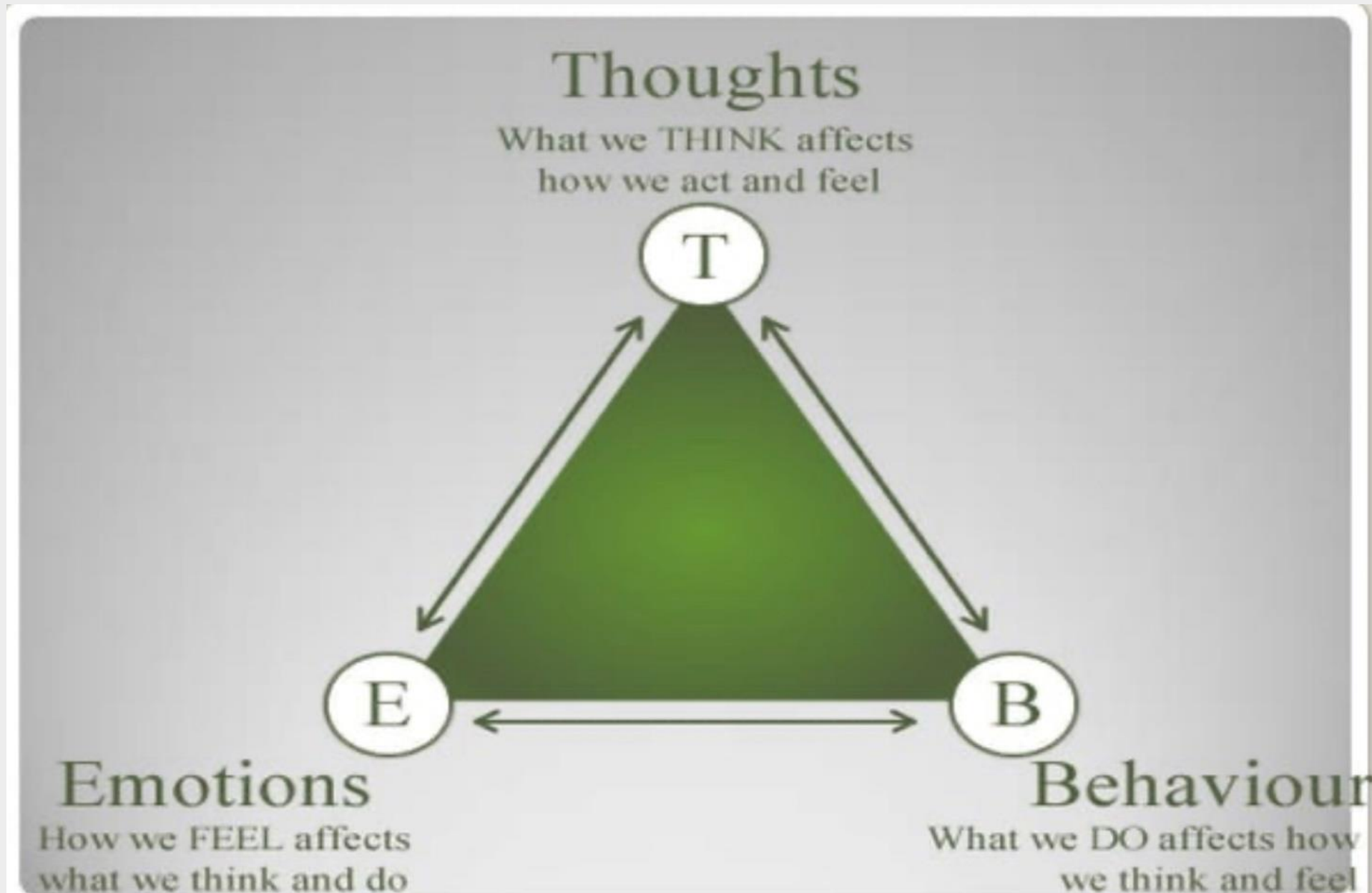
Based on the theory that a person's effect and behaviour are largely determined by the way in which he cognitively structures and interprets the world(cognitive schemata developed from previous experience).

When faced with any situation, we respond with certain immediate interpretations (called automatic thoughts) which are highly influenced by our underlying or hidden beliefs or assumptions.

AIM OF COGNITIVE-BEHAVIORAL THERAPY

To identify and correct cognitive distortions (automatic thoughts and underlying beliefs) and maladaptive behaviors that result from them

Through a combination of verbal and behaviour modification techniques



TECHNIQUES OF CBT

A- COGNITIVE (VERBAL) TECHNIQUES

1. Identify and test automatic thoughts, i.e. test their rationality as hypotheses rather than facts.
2. Identify and test the underlying assumptions or core beliefs.
3. Correcting the distorted cognitions and replacing them with positive and more adaptive cognitive habits.
4. Rehearsal of the new cognitive and behavioral responses.

TECHNIQUES OF CBT

B- BEHAVIORAL TECHNIQUES

Methods including:

1. Activity scheduling
2. Graded task assignment (graded social activity to correct social withdrawal)
3. Rehearsal of new behaviour
4. Rating of progress in the amount of mastery and pleasure.
5. Diversion techniques: physical activity and exercise, work, social contact.

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CHARACTERISTIC FEATURES OF CBT

1. Duration

short-term and time-limited, usually 15-20 sessions, over 3 months

2. Therapist role

active, directive, understanding and empathetic

3. Focused on

a. conscious aspects of experience and behaviour.

B. current (here and now problems)

CHARACTERISTIC FEATURES OF CBT, CONTD.

4. **Structured**

- a. problems and goals operationally defined.
- B. agenda prepared for each session.

5. **Format**

usually individual but can be used in other formats.

INDICATIONS OF CBT

- 1. Depressive disorder (non-psychotic)**
- 2. Anxiety disorders
ex. Panic, phobic and generalized.**
- 3. Obesity and eating disorders.**
- 4. Substance related disorders.**

GROUP PSYCHOTHERAPY

A form of therapy in which therapeutic changes occur as a result of the interactions of patients with other patients and at least one trained professional therapist in a group setting.



GROUP PSYCHOTHERAPY/ GOALS

- 1. Relief of symptoms**
- 2. Resolution of intrapsychic and interpersonal problems through insight and corrective experiences.**
- 3. Encouraging personality growth and development.**

GROUP PSYCHOTHERAPY/ CHARACTERISTICS

➤ **Size:**

6-12 patients (optimum 8)

➤ **Time:**

1-2 hours once or twice weekly

➤ **Duration:**

months to years depending on goals and therapy model.

GROUP PSYCHOTHERAPY/ TYPE OF PATIENTS

- **Heterogeneous:**
(different diagnoses)
- **Homogeneous:**
(single diagnosis, ex. Special groups for substance abuse or PTSD)

GROUP PSYCHOTHERAPY/ MODEL OF THERAPY

- Supportive
- Psychodynamic
- Experiential
- Cognitive and behavioral models
- Interactional model (Yalom)
- Transactional analysis (Bern)
- psychodrama

GROUP PSYCHOTHERAPY/ INDICATIONS

Almost all diagnoses, except:

- Acute manic
- Acute psychotic episode
- Antisocial personality disorder



FAMILY THERAPY

Family therapy is based on the idea that the patient's psychological disturbance reflects a significant disturbance in his family

Ex. Disturbed patterns of behaviors and relations among his family members.

FAMILY THERAPY

Therapy aims at helping family members gain insight into their problems and disturbed relational patterns and change their dysfunctional behavior and emotions into healthier patterns.

The focus of therapy is on family as a whole rather than the individual patient.

The family is viewed as a system suffering from dysfunctions resulting from disturbed rules, roles and relations among its members.



MARITAL THERAPY

A form of therapy concerned with maladjusted marital couples.

It aims at resolving interpersonal and related intrapsychic individual problems of the couple.

The therapy is focused on the “relationship” rather than any of the individual partners.



REHABILITATION

It's is the management of disabilities and handicaps resulting from a psychiatric disorder conducted by trained therapists.

It is particularly important in chronic psychiatric disorders (such as chronic schizophrenia and mood disorders)

REHABILITATION

Aiming to improve patient performance in different functional domains including:

1. Personal hygiene and self-care skills.
2. Social and interpersonal skills.
3. Vocational (work) and/or educational skills (occupational therapy)
4. Recreation and activity therapies, ex. Art, music, sports, celebration of social events.

MILIEU THERAPY

An environment that is structured (organized) to assist patients to:

1. Control and modify problematic (maladaptive) behaviors.
 2. Promote adaptive psychosocial skills in coping with the self, others and environment.
- The process typically combines the benefits of group therapy and an individualized treatment approach.



MILIEU THERAPY

The focus is on social relationships as well as occupational and recreational activities.

To achieve its goals it uses different therapeutic modalities particularly:

1. Group therapies
2. Different rehabilitative techniques
3. Structured activities of daily living for all patients.



TEAM INVOLVED IN MILIEU THERAPY

Therapy is based on **the coordinated work of a multidisciplinary team** which includes:

- The psychiatrist
- Psychologist
- Psychiatric nursing staff
- Social workers
- Occupational/ art/ play and recreational therapists

PLACES OF MILIEU THERAPY

It can be established in different locations, for example:

- Psychiatric hospitals
- Psychiatric patient ward of a general hospital
- Day hospital
- Special houses for geriatric patients.
- Etc...